

New classification

Table 2 shows the new international classification approved by the EADV Psychodermatology Task Force, ESDaP, and APMNA. The term "disorder" is the preferred terminology for both mental health and dermatological diagnoses. Two main groups are recognized based on pathophysiological and phenomenological similarities. In both groups, patients may develop secondary psychiatric disorders—either as a result of the psychosocial burden of skin disease or due to an underlying psychiatric condition. These secondary psychiatric comorbidities (e.g., depression, anxiety, sleep disorders) are not explicitly listed in the table, as they are common across both main groups.

Primary mental health disorders affecting the skin

This group includes psychodermatological disorders where psychiatric or psychosocial factors are central to the etiology, with skin symptoms or lesions occurring secondarily. The primary psychiatric conditions underlying each disorder are indicated in the table (e.g., psychotic disorders), although comorbid psychiatric conditions (such as anxiety disorders) may also be present and contribute to symptom severity.

This group is subdivided into two subcategories based on the presence or absence of visible skin lesions. While these distinctions serve as general diagnostic guidelines, they are not absolute.

Subgroups

1. Psychotic disorders

The prototypical example is delusional disorder, somatic type, most commonly presenting as delusional infestation. Patients may exhibit excoriations, erosions, or ulcers due to skin picking or scratching in response to false beliefs of infestation. However, some individuals do not

develop visible skin lesions. Therefore, this disorder is classified in both subgroups—those with and without visible skin lesions.

2. **Obsessive-compulsive and related disorders (OCDs)**

Includes conditions such as body-focused repetitive behaviors (BFRBs), body dysmorphic disorder (BDD), and tanorexia.

- **BFRBs** (e.g., skin picking disorder, trichotillomania) are characterized by self-inflicted skin lesions.

- **Body dysmorphic disorder:** Skin picking is recognized as a repetitive behavior performed in response to appearance concerns, as defined in DSM-5-TR:

- > "At some point during the course of the disorder, the individual has performed repetitive behaviors (e.g., mirror checking, excessive grooming, skin picking, reassurance seeking) or mental acts (e.g., comparing his or her appearance with that of others) in response to the appearance concerns."

- **Tanorexia:** Pathological tanning behavior leads to skin hyperpigmentation, classified as a behavioral addiction within OCDs.

3. **Non-substance-related addictive disorders**

Includes disorders such as compulsive skin picking or excessive cosmetic procedures driven by appearance concerns, often overlapping with OCDs.

4. **Non-suicidal self-inflicted disorders (NSSIDs)**

Encompasses deliberate self-harm without suicidal intent, resulting in skin injuries such as cuts, burns, or abrasions.

5. **Somatic symptom and related disorders**

Includes conditions where excessive thoughts, feelings, or behaviors are related to somatic symptoms—in this context, skin-related concerns—despite minimal or no dermatological findings.

Primary skin disorders linked with mental health

This group includes primary dermatological conditions with multifactorial etiology, where psychological stress or psychiatric comorbidities play a significant role in onset, exacerbation, or maintenance of the skin disease.

Examples include psoriasis, atopic dermatitis, chronic urticaria, and prurigo nodularis.

This group is also subdivided into: - Disorders with visible skin lesions - Disorders without visible skin lesions (e.g., chronic pruritus)

Skin lesions: Primary vs. secondary

- **Primary skin lesions**

Arise de novo and are characteristic of the initial stage of a skin disease (e.g., erythematous plaques in psoriasis, vesicles in herpes zoster). These are not typically seen in primary mental health disorders affecting the skin.

- **Secondary skin lesions**

Develop over time due to disease progression, external manipulation, or pathological behaviors. They include scales, crusts, erosions, ulcers, scars, lichenification, and acquired localized hypertrichosis (from repeated friction).

Secondary lesions can occur in both main groups:

- In **primary mental health disorders**, they result from self-inflicted trauma (e.g., excoriations in delusional infestation or BFRBs).
- In **primary skin disorders linked with mental health**, they may be worsened by stress-induced behaviors (e.g., scratching in atopic dermatitis or prurigo).

Thus, while primary lesions are specific to intrinsic skin diseases, secondary lesions are transdiagnostic and reflect behavioral or progressive changes, commonly observed across both psychodermatological categories.